

Sender information: user@example.com

To: John Doe <john.doe@example.com>

 Subject 1: How do you decide on a treatment for patients with pretreated HR+/HER2– mBC?

Subject 2: Get insights on treating patients with pretreated HR+/HER2– mBC

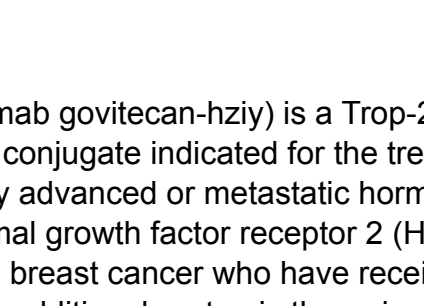
Subject 3: Looking for a different approach for your patients with pretreated HR+/HER2– mBC?

Preheader 1: Get insights from your peers to help your patients.

Preheader 2: Gain a new perspective from your colleagues in an exclusive video.

Preheader 3: Get insights from your peers in exclusive peer-to-peer videos.

Please see full [Prescribing Information](#), including **BOXED WARNING** for Neutropenia and Diarrhea, and Important Safety Information below.



Valuable insights from your peers for your patients with pretreated HR+/HER2– mBC

INDICATION

TRODELVY® (sacituzumab govitecan-hziy) is a Trop-2-directed antibody and topoisomerase inhibitor conjugate indicated for the treatment of adult patients with unresectable locally advanced or metastatic hormone receptor (HR)-positive, human epidermal growth factor receptor 2 (HER2)-negative (IHC 0, IHC 1+ or IHC 2+/ISH–) breast cancer who have received endocrine-based therapy and at least two additional systemic therapies in the metastatic setting.

[Dear] [Dr., Mr., Mrs., Ms., Prof., blank] [LastName],

[I am looking forward to meeting soon.]

If you're looking to keep up with treatment trends, you may be interested in these exclusive peer-to-peer videos. They contain insights on prescribing an ADC like TRODELVY for patients with pretreated HR+/HER2– mBC.¹

IMPORTANT SAFETY INFORMATION

BOXED WARNING: NEUTROPENIA AND DIARRHEA

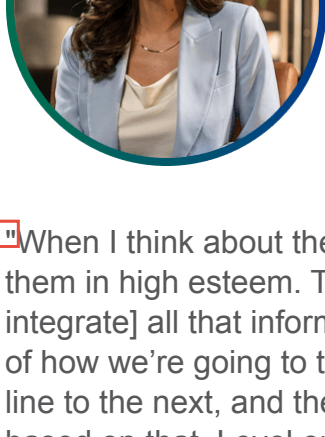
- **TRODELVY can cause severe, life-threatening, or fatal neutropenia. Withhold TRODELVY for absolute neutrophil count below 1500/mm³ or neutropenic fever. Monitor blood cell counts periodically during treatment. Primary prophylaxis with G-CSF is recommended for all patients at increased risk of febrile neutropenia. Initiate anti-infective treatment in patients with febrile neutropenia without delay.**
- **TRODELVY can cause severe diarrhea. Monitor patients with diarrhea and give fluid and electrolytes as needed. At the onset of diarrhea, evaluate for infectious causes and, if negative, promptly initiate loperamide. If severe diarrhea occurs, withhold TRODELVY until resolved to ≤Grade 1 and reduce subsequent doses.**

Please see additional Important Safety Information below.

Learn more about the TRODELVY safety profile by clicking [here](#).

TROPiCS-02 Study

What was the response to the TROPiCS-02 data from prominent physicians?



 I believe this was a well-needed trial in a later-line setting, and the trial design really reflected the patients that we are seeing.

— Martin Dietrich, MD, PhD

TROPiCS-02 Study Design^{1,2}

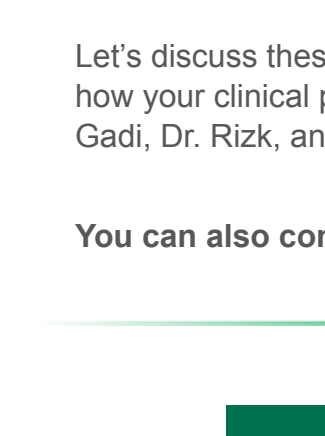
TROPiCS-02 was a Phase 3, randomized, active-controlled, open-label study (N=543) in patients with HR+/HER2– mBC who were previously treated with ≥1 endocrine therapy, a CDK4/6i, and a taxane in any setting, and who had received 2 to 4 lines of chemotherapy in the metastatic setting. Patients were randomized (1:1) to receive TRODELVY 10 mg/kg as an IV infusion on Days 1 and 8 of a 21-day cycle (n=272) or investigator's choice of single-agent chemotherapy (n=271), which included eribulin, vinorelbine, gemcitabine, or capecitabine. Patients were treated until disease progression or unacceptable toxicity.

Learn more about the TROPiCS-02 results for primary and secondary endpoints. Get more insights on what it means for your patients and clinical practice.

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NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®)

What is being said about the guidelines from prominent physicians?



 For many oncologists, I think the National Comprehensive Cancer Network® (NCCN®) is the gold standard for understanding what treatment options [to] recommend.

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 When I think about the NCCN, I really do hold them in high esteem. They look at the data...[and integrate] all that information, putting it in the flow of how we're going to treat our patients from one line to the next, and then giving recommendations based on that. Level one evidence is very hard to get.

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NCCN recommendation for sacituzumab govitecan-hziy (TRODELVY)

The first and only Trop-2–directed ADC designated as an NCCN Category 1 preferred regimen in pretreated HR+/HER2– mBC^{1,3}

NCCN: Sacituzumab govitecan-hziy (TRODELVY) is recommended as a Category 1 preferred treatment option for adult patients with unresectable locally advanced or metastatic HR+/HER2– breast cancer who have received prior treatment, including endocrine therapy, a CDK4/6 inhibitor, and at least 2 lines of chemotherapy (one including a taxane), at least one of which was in the metastatic setting. It may be considered for later line if not used as second-line therapy.^{2,a}

^aIf not a candidate for fam-trastuzumab deruxtecan-nxki.³

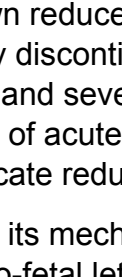
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Category 1 indicates that based upon high-level evidence (≥1 randomized phase 3 trials or high-quality, robust meta-analyses), there is uniform NCCN consensus (≥85% support of the panel) that the intervention is appropriate.³

The NCCN recommendation differs from the sacituzumab govitecan-hziy (TRODELVY) Prescribing Information.

Get more insights from your peers regarding the NCCN Guidelines and what factors impact treatment decisions.

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You can also contact me at [\[user phone\]](#).

Schedule a time with me here >

IMPORTANT SAFETY INFORMATION (CONT'D)

CONTRAINDICATIONS

- Severe hypersensitivity reaction to TRODELVY.

WARNINGS AND PRECAUTIONS

Neutropenia: Severe, life-threatening, or fatal neutropenia can occur as early as the first cycle of treatment and may require dose modification. Neutropenia occurred in 64% of patients treated with TRODELVY. Grade 3-4 neutropenia occurred in 49% of patients. Febrile neutropenia occurred in 6%. Neutropenic colitis occurred in 1.4%. Primary prophylaxis with G-CSF is recommended starting in the first cycle of treatment in all patients at increased risk of febrile neutropenia, including older patients, patients with previous neutropenia, poor performance status, organ dysfunction, or multiple comorbidities. Monitor absolute neutrophil count (ANC) during treatment. Withhold TRODELVY for ANC below 1500/mm³ on Day 1 of any cycle or below 1000/mm³ on Day 8 of any cycle. Withhold TRODELVY for neutropenic fever. Treat neutropenia with G-CSF and administer prophylaxis in subsequent cycles as clinically indicated or indicated in Table 2 of USPI.

Diarrhea: Diarrhea occurred in 64% of all patients treated with TRODELVY. Grade 3-4 diarrhea occurred in 11% of patients. One patient had intestinal perforation following diarrhea. Diarrhea that led to dehydration and subsequent acute kidney injury occurred in 0.7% of all patients. Withhold TRODELVY for Grade 3-4 diarrhea and resume when resolved to ≤ Grade 1. At onset, evaluate for infectious causes and if negative, promptly initiate loperamide, 4 mg initially followed by 2 mg with every episode of diarrhea for a maximum of 16 mg daily. Discontinue loperamide 12 hours after diarrhea resolves. Additional supportive measures (e.g., fluid and electrolyte substitution) may also be employed as clinically indicated. Patients who exhibit an excessive cholinergic response to treatment can receive appropriate premedication (e.g., atropine) for subsequent treatments.

Hypersensitivity and Infusion-Related Reactions: TRODELVY can cause serious hypersensitivity reactions including life-threatening anaphylactic reactions. Severe signs and symptoms included cardiac arrest, hypotension, wheezing, angioedema, swelling, pneumonitis, and skin reactions. Hypersensitivity reactions within 24 hours of dosing occurred in 35% of patients. Grade 3-4 hypersensitivity occurred in 2% of patients. The incidence of hypersensitivity reactions leading to permanent discontinuation of TRODELVY was 0.2%. The incidence of anaphylactic reactions was 0.2%. Pre-infusion medication is recommended. Have medications and emergency equipment to treat such reactions available for immediate use. Observe patients closely for hypersensitivity and infusion-related reactions during each infusion and for at least 30 minutes after completion of each infusion. Permanently discontinue TRODELVY for Grade 4 infusion-related reactions.

Nausea and Vomiting: TRODELVY is emetogenic and can cause severe nausea and vomiting. Nausea occurred in 64% of all patients treated with TRODELVY and Grade 3-4 nausea occurred in 3% of these patients. Vomiting occurred in 35% of patients and Grade 3-4 vomiting occurred in 2% of these patients. Premedicate with a two or three drug combination regimen (e.g., dexamethasone with either a 5-HT3 receptor antagonist or an NK₁ receptor antagonist as well as other drugs as indicated) for prevention of chemotherapy-induced nausea and vomiting (CINV). Withhold TRODELVY doses for Grade 3 nausea or Grade 3-4 vomiting and resume with additional supportive measures when resolved to Grade ≤ 1. Additional antiemetics and other supportive measures may also be employed as clinically indicated. All patients should be given take-home medications with clear instructions for prevention and treatment of nausea and vomiting.

Increased Risk of Adverse Reactions in Patients with Reduced UGT1A1 Activity: Patients homozygous for the uridine diphosphate-glucuronosyl transferase 1A1 (UGT1A1)*28 allele are at increased risk for neutropenia, febrile neutropenia, and anemia and may be at increased risk for other adverse reactions with TRODELVY. The incidence of Grade 3-4 neutropenia was 58% in patients homozygous for the UGT1A1*28, 49% in patients heterozygous for the UGT1A1*28 allele, and 43% in patients homozygous for the wild-type allele. The incidence of Grade 3-4 anemia was 21% in patients homozygous for the UGT1A1*28 allele, 10% in patients heterozygous for the UGT1A1*28 allele, and 9% in patients homozygous for the wild-type allele. Closely monitor patients with known reduced UGT1A1 activity for adverse reactions. Withhold or permanently discontinue TRODELVY based on clinical assessment of the onset, duration and severity of the observed adverse reactions in patients with evidence of acute early-onset or unusually severe adverse reactions, which may indicate reduced UGT1A1 function.

Embryo-Fetal Toxicity: Based on its mechanism of action, TRODELVY can cause teratogenic and/or embryo-fetal lethality when administered to a pregnant woman. TRODELVY contains a genotoxic component, SN-38, and targets rapidly dividing cells. Advise pregnant women and females of reproductive potential of the potential risk to a fetus. Advise females of reproductive potential to use effective contraception during treatment with TRODELVY and for 6 months after the last dose. Advise male patients with female partners of reproductive potential to use effective contraception during treatment with TRODELVY and for 3 months after the last dose.

ADVERSE REACTIONS

In the pooled safety population, the most common (≥25%) adverse reactions including laboratory abnormalities were decreased leukocyte count (84%), decreased neutrophil count (75%), decreased hemoglobin (69%), diarrhea (64%), nausea (64%), decreased lymphocyte count (63%), fatigue (51%), alopecia (45%), constipation (37%), increased glucose (37%), decreased albumin (35%), vomiting (35%), decreased appetite (30%), decreased creatinine clearance (28%), increased alkaline phosphatase (28%), decreased magnesium (27%), decreased potassium (26%), and decreased sodium (26%).

In the TROPiCS-02 study (locally advanced or metastatic HR-positive, HER2-negative breast cancer), the most common adverse reactions (incidence ≥25%) were diarrhea, fatigue, nausea, alopecia, and constipation. The most frequent serious adverse reactions (SAR) (>1%) were diarrhea (5%), febrile neutropenia (4%), neutropenia (3%), abdominal pain, colitis, neutropenic colitis, pneumonia, and vomiting (each 2%). SAR were reported in 28% of patients, and 6% discontinued therapy due to adverse reactions. The most common Grade 3-4 lab abnormalities (incidence ≥25%) in the TROPiCS-02 study were reduced neutrophils and leukocytes.

DRUG INTERACTIONS

- UGT1A1 Inhibitors:** Concomitant administration of TRODELVY with inhibitors of UGT1A1 may increase the incidence of adverse reactions due to potential increase in systemic exposure to SN-38. Avoid administering UGT1A1 inhibitors with TRODELVY.
- UGT1A1 Inducers:** Exposure to SN-38 may be reduced in patients concomitantly receiving UGT1A1 enzyme inducers. Avoid administering UGT1A1 inducers with TRODELVY.

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[Thank you for your time. Please reach out with any questions about TRODELVY or how TRODELVY may be right for your appropriate patients.]

[user photo]

[Sincerely],

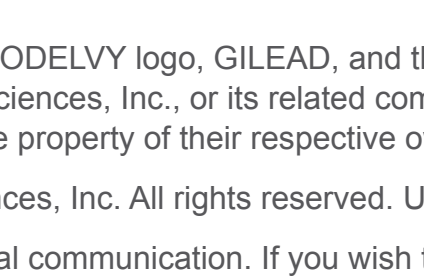
[user name]

[user email address]

[user phone]

ADC=antibody-drug conjugate; CDK4/6=cyclin-dependent kinase 4/6; CDK4/6i=cyclin-dependent kinase 4/6 inhibitor; HER2=human epidermal growth factor receptor 2-negative; HR+=hormone receptor-positive; IHC=immunohistochemistry; IV=intravenous; mBC=metastatic breast cancer; NCCN=National Comprehensive Cancer Network; Trop-2=trophoblast cell-surface antigen-2.

References: 1. TRODELVY. Prescribing Information. Gilead Sciences, Inc.; March 2025. 2. Ruqo HS, Bardia A, Marmé F, et al. Sacituzumab govitecan in hormone receptor–positive/human epidermal growth factor receptor 2–negative metastatic breast cancer. *J Clin Oncol*. 2022;40(29):3365–3376. 3. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®) for Breast Cancer V.5.2025. © National Comprehensive Cancer Network, Inc. 2025. All rights reserved. Accessed October 20, 2025. To view the most recent and complete version of the guidelines, go online to NCCN.org.



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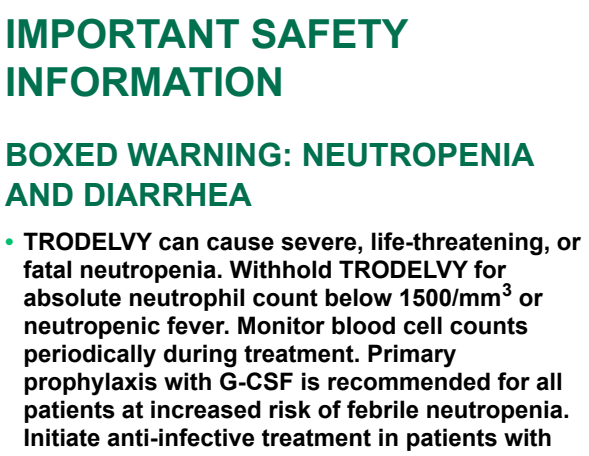
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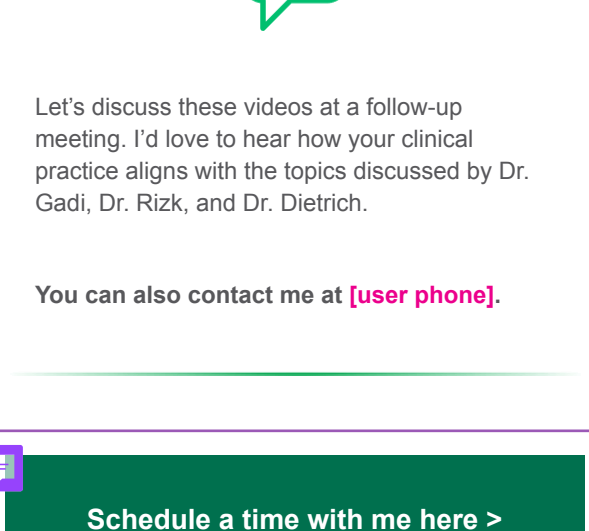
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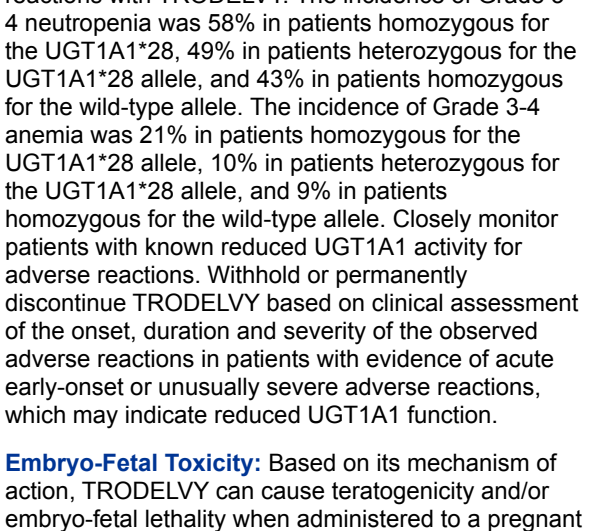
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IMPORTANT SAFETY INFORMATION (CONT'D)

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- Severe hypersensitivity reaction to TRODELVY.

WARNINGS AND PRECAUTIONS

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Embryo-Fetal Toxicity: Based on its mechanism of action, TRODELVY can cause teratogenicity and/or embryo-fetal lethality when administered to a pregnant woman. TRODELVY contains a genotoxic component, SN-38, and targets rapidly dividing cells. Advise pregnant women and females of reproductive potential of the potential risk to a fetus. Advise females of reproductive potential to use effective contraception during treatment with TRODELVY and for 6 months after the last dose. Advise male patients with female partners of reproductive potential to use effective contraception during treatment with TRODELVY and for 3 months after the last dose.

ADVERSE REACTIONS

In the pooled safety population, the most common (≥25%) adverse reactions including laboratory abnormalities were decreased leukocyte count (84%), decreased neutrophil count (75%), decreased hemoglobin (69%), diarrhea (64%), nausea (64%), decreased lymphocyte count (63%), fatigue (51%), alopecia (45%), constipation (37%), increased glucose (37%), decreased albumin (35%), vomiting (35%), decreased appetite (30%), decreased creatinine clearance (28%), increased alkaline phosphatase (28%), decreased magnesium (27%), decreased potassium (26%), and decreased sodium (26%).

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DRUG INTERACTIONS

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[Thank you for your time. Please reach out with any questions about TRODELVY or how TRODELVY may be right for your appropriate patients.]

[user photo]

[Sincerely],

[user name]

[user email address]

[user phone]

ADC=antibody-drug conjugate; CDK4/6=cyclin-dependent kinase 4/6; CDK4/6i=cyclin-dependent kinase 4/6 inhibitor; HER2=human epidermal growth factor receptor 2–negative; HR+=hormone receptor-positive; IHC=immunohistochemistry; IV=intravenous; mBC=metastatic breast cancer; NCCN=National Comprehensive Cancer Network; Trop-2=trophoblast cell-surface antigen-2.

References: 1. TRODELVY. Prescribing Information. Gilead Sciences, Inc.; March 2025. 2. Ruogo HS, Bardia A, Marmé F, et al. Sacituzumab govitecan in hormone receptor–positive/human epidermal growth factor receptor 2–negative metastatic breast cancer. *J Clin Oncol*. 2022;40(29):3365–3376. 3. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®) for Breast Cancer V.5.2025.

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